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20 October 2026

Selection Panel
Metropolitan Public Health Service
Melbourne, VIC

Re: Senior Evaluation and Data Quality Coordinator

Dear Selection Panel,

I am writing to apply for the Senior Evaluation and Data Quality Coordinator position. My work has focused on making service data dependable enough for practical decisions while keeping the people represented by those records in view. I have worked with clinical teams, managers, analysts, and privacy colleagues to define measures, test assumptions, and explain uncertainty before a report is circulated.

At a fictional regional health network, I inherited a reporting process that depended on manual spreadsheet corrections. The first task was to understand why teams had made those corrections rather than replacing them blindly. I interviewed the people who prepared and used the report, compared the documented rules with actual extracts, and built a repeatable sequence of checks. Each exception was logged for review. The revised process shortened preparation time, but the more valuable outcome was that staff could trace each number to its source and see where a definition had changed.

I then helped evaluate a new referral pathway. We agreed on a simple analysis plan before looking at results, including the cohort, time window, exclusions, and the measures that would be useful to service leaders. Some records arrived late and some service codes changed during the pilot. I documented those issues in the published report and provided a comparable series beside the operational view. This let the panel discuss a plausible service effect without treating an imperfect observational comparison as proof. I also wrote a short explanation for clinicians who did not need to read the full technical appendix.

The role appeals to me because it combines careful analysis with the relationships needed to use it well. I would bring patient collaboration, practical SQL and Python experience, and a willingness to revise a measure when a stakeholder finds that it does not answer the question they actually face. I value concise reporting, but I do not shorten away limitations that matter to the decision. If appointed, I would begin by learning how your teams currently define quality and where they most often need to reconcile conflicting data.

In a later project, I worked with a small team to review reporting for a multi-site outreach program. Staff had good reasons to use local categories, but those categories did not always align with the central extract. I convened a short series of meetings to understand the differences, wrote a mapping that preserved the original values, and tested it on records that each site identified as difficult. We published the mapping and its limits with the report. The result was a more useful shared view without pretending that every local service delivered care in exactly the same way.

I also enjoy building capability around the work. I have prepared guides that walk a colleague from a source extract to a final figure, including checks for missing values, small groups, and unexpected changes. In peer reviews I ask someone else to repeat the analysis from the written method and tell me where the instructions are unclear. That habit has caught errors and improved handover. I would bring the same practical approach to your evaluation program, working with staff who use the outputs as well as those who prepare them.

I have learned that a useful evaluation process must remain understandable after the original project team moves on. In one handover, I asked an analyst who had not seen the work to reproduce a prior reporting period from our notes. The exercise revealed assumptions about late-arriving records and a category that had changed name. We amended the guide, added a check that would catch the issue automatically, and explained the correction to the service manager before publishing the next report. That small investment prevented the team from repeating a misleading comparison.

Your service would also benefit from my experience communicating with mixed audiences. I have presented results to clinicians who want to know whether a finding changes their next conversation with a patient, and to managers who need a stable monthly measure. I prepare one clear account of the question and evidence, then provide the technical method for those who need to inspect it. When a figure is uncertain, I say why, how much it may matter, and what information would help resolve it. I believe that is more valuable than a polished chart that looks definitive but cannot withstand a careful question.

I would approach the first months in this role by learning your current reporting calendar and identifying which decisions each recurring product is meant to support. I would ask where staff spend time reconciling figures, which definitions are debated most often, and which limitations are repeated in meetings but absent from the written report. From there, I would prioritise a small set of changes that can be tested with the people who use the information. A definition that is consistent but irrelevant is no better than a fast report that cannot be reproduced.

I am comfortable working across teams that have different levels of technical detail. In workshops I use concrete records to test a proposed measure, then write the rule in ordinary language before translating it into code. When two interpretations are reasonable, I record the decision and the alternative so that a later review can revisit it. I also build space for staff to challenge a result after publication. That feedback is often the quickest way to discover a missing service context or an upstream change that automated checks could not anticipate.

I would bring the same discipline to prioritising work. When a reporting problem is raised, I first ask which decision it affects, how many people rely on the measure, and whether the current result could mislead them. I then agree on a small test with the service owner and analyst: a sample of source records, the expected rule, and the checks needed before release. This avoids a long redesign when a clear correction will do, while making sure a larger structural issue is not dismissed as a one-off error.

My approach to handover is equally practical. I leave a record of source versions, decisions about inclusion, known exceptions, and the person responsible for the next review. I ask a colleague to reproduce one prior result from that record and note where the instructions are unclear. If a definition changes, I preserve the earlier version and explain the effect on comparisons. I would use that process to help your team maintain useful reports as systems and staff change.

Thank you for considering my application. I would welcome a conversation about how I could help your teams build trustworthy evaluation and reporting practices.

Kind regards,
Jordan Rivera-McAllister